Arolygiaeth Gofal Cymru
Care Inspectorate Wales

Inspection Report

Parkview House

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The inspection visits for this service took place between 14/09/2026 and 17/09/2026

Service Information:

Operated by:	Parkview Residential (Swansea) Limited
Care Type:	Care Home Service Adults Without Nursing
Registered places:	9
Main language(s):	English
Promotion of Welsh language and culture:	The provider is not promoting the Welsh language and culture needs of people, and this requires improvement.

Ratings:

Well-being	Good
Care & Support	Good
Environment	Good
Leadership & Management	Good

Summary:

Well-being is rated good because people are supported to live healthy, safe and meaningful lives with choice and control over daily decisions. People told the inspector they were happy living at the service. Individuals access community activities, maintain relationships and are supported through positive risk-taking. No safeguarding concerns, notifications or open compliance issues were identified. Areas for development include strengthening the Welsh language active offer and evidencing activities through photographs and displays.

Care and Support is rated good because people receive person-centred care supported by current care planning, risk management and multidisciplinary involvement. Safeguarding, infection prevention and activity provision were compliant. Medication systems were generally effective with regular audits in place; however, medication recording issues indicate the provider should continue to demonstrate audit actions are reducing risk.

Environment is rated good because accommodation is suitable, well maintained and supports people's well-being outcomes. Most fire safety actions had been completed, with remaining works subject to clear plans and timescales.

Leadership and Management is rated good because governance, quality assurance and Care Staff development arrangements were effective. However, significant reliance on agency Care Staff requires ongoing monitoring to maintain continuity of care. Overall, the service was meeting expectations and achieving positive outcomes for people.

Findings:

Well-being — Good

People are supported to live healthy and meaningful lives with a good degree of choice and control. Evidence showed people are involved in decisions affecting their daily lives, supported to manage their finances where appropriate, and encouraged to take positive risks that promote independence and community participation. Care planning and risk management arrangements were found to support personal outcomes and well-being.

People appeared safe and protected from abuse and neglect. Inspection planning and analysis identified no open safeguarding matters, concerns, notifications, non-compliance notices, or important notices associated with the service. Systems were in place to ensure concerns are identified and responded to appropriately, and safeguarding arrangements were assessed as compliant.

People are supported to maintain relationships and take part in activities that matter to them. Individual activity planners are used, with activities tailored to people's interests and preferences. Opportunities included gym attendance, swimming, walking, community outings, participation in daily household tasks, access to transport, and support to maintain contact with family and friends. Care Staff facilitated activities on an individual basis, and people were observed to be engaged and content.

Feedback available to the inspection was positive. At the previous inspection, people using the service commented, "*It's good here, I like it*," while relatives described the service as "*home from home*" and reported positive changes in people's well-being. During this inspection, people told the inspector they were happy living at the service. Visiting professionals also reported that Care Staff knew people well and provided caring support.

People live in accommodation that supports their well-being outcomes. Evidence indicated that accommodation meets people's needs, appropriate equipment is available, and people's views are considered when improvements or adaptations are made to the environment.

Opportunities exist to further evidence and celebrate activities through photographs and displays, and to strengthen the Welsh language active offer should future needs arise. Residents' meetings were not taking place, although individual consultation arrangements were in place.

People experience positive well-being outcomes and are supported to live safely, maintain relationships, engage in meaningful activities, and exercise choice and control over their lives. The evidence provides good assurance that well-being outcomes are being achieved.

Care & Support — Good

People receive person-centred care and support which promotes their wellbeing, safety and independence. People told us they were happy living at the service, felt safe and said Care Staff supported them in the way they wanted. Care records demonstrated that people's assessed needs, preferences, communication needs, health requirements and risks were considered when planning and reviewing care.

Care plans, risk assessments, health records and reviews were generally current and reflected people's individual needs. Case tracking demonstrated good multidisciplinary involvement, timely healthcare referrals, regular reviews, monitoring of outcomes and personalised support planning. People were supported to participate in activities that mattered to them, including community access, exercise, hobbies and maintaining important relationships. Care Staff knew people well and described person-centred approaches tailored to individual needs and behaviours.

Safeguarding arrangements were established and Care Staff demonstrated a good understanding of safeguarding responsibilities, reporting procedures and escalation routes. Care Staff received safeguarding training and were aware of current risks affecting people they supported. Safeguarding referrals, incidents and behavioural risks were reviewed through governance processes and evidence showed learning from incidents resulted in changes to support arrangements and risk management approaches. A previous safeguarding concern relating to restrictive practice was investigated and concluded with no further action following multidisciplinary review.

Most medication management arrangements were effective. Medication policies, audits, stock controls, storage arrangements and Care Staff training were in place. Monthly medication audits demonstrated ongoing oversight. However, the inspection identified controlled drug recording discrepancies, missing signatures and evidence of previous medication errors. While corrective actions had been taken, recurring medication audit findings indicate the provider should strengthen assurance that audit actions are effectively reducing medication risks.

The service was found to be compliant with infection prevention and control requirements. Appropriate PPE was available and used correctly, waste management arrangements were suitable, cleaning systems were in place and Care Staff demonstrated an appropriate understanding of infection control practice. The environment was observed to be clean and organised, supporting people to live in a safe and hygienic environment.

People receive care and support that is personalised, responsive and focused on achieving positive outcomes. Strong Care Staff knowledge, person-centred planning, safeguarding arrangements and multidisciplinary working provide positive outcomes for people.

Environment

Good

People live in an environment that generally supports their well-being and independence. People told us they were happy living at the service, liked their rooms and friendships, felt safe and were supported by Care Staff in the way they preferred. Bedrooms viewed were personalised, clean and reflected people's needs and preferences. The home was welcoming, clean and homely, with suitable communal areas, equipment and facilities that supported positive outcomes.

The provider has maintained accommodation that is largely suitable for the people living at the service. Internal communal areas were clean, accessible and appropriately furnished. People had access to transport, activities, outdoor space and equipment that promoted choice, independence and community involvement. Essential servicing arrangements, maintenance systems and health and safety checks were in place.

However, several areas required improvement. External areas appeared untidy, with damaged fencing, overgrown grounds, cluttered storage areas and temporary security arrangements. The conservatory roof was leaking and presented a potential electrical hazard, requiring immediate action. The provider has a two-year improvement plan to address refurbishment and maintenance needs.

The provider has made significant progress in addressing fire safety concerns identified within the January 2026 Fire Risk Assessment. Most actions had been completed or substantially progressed, including improvements to compartmentation, signage, evacuation arrangements, fire safety monitoring and emergency planning. This provides greater assurance that people are protected from foreseeable risks.

Despite this progress, some important risks remained. Fire door remedial works had not yet been completed and remained the most significant outstanding life-safety issue. Additional fire safety actions, including aspects of smoke ventilation and fire safety assurance, remained ongoing. Fire records were not always consistently maintained and further verification is required to confirm all remedial works have been completed to an appropriate standard. The inspection also identified general maintenance issues affecting the external environment and parts of the premises.

People live in a safe and generally suitable environment which supports their well-being outcomes. The provider has demonstrated substantial improvement in fire safety and environmental oversight. However, outstanding fire safety actions, maintenance issues and the condition of external areas require continued attention to ensure the environment consistently meets expectations and supports people's safety and well-being.

Leadership & Management — Good

Leadership and management arrangements support positive outcomes for people. People said they felt safe, were supported as they wished and were happy with their accommodation. Care Staff described a positive, person-centred culture and said they understood their roles, knew people well and felt valued, trained and supported. They consistently reported that managers were accessible, concerns were acted upon and Care Staffing levels enabled them to provide good care.

Clear management and Responsible Individual (RI) oversight were evident. Policies, Care Staff records, recruitment checks, supervision and appraisal arrangements were generally compliant. Governance systems reviewed incidents, safeguarding, complaints, medication, Care Staffing and service improvement. Leaders responded to identified risks, including escalating safeguarding matters and taking immediate action where environmental hazards were identified.

A structured framework included RI statutory visits, Quality of Care Review reports, audits, governance committees, risk registers, KPIs and electronic monitoring systems. Evidence showed improvement in cleanliness, Care Staff supervision, training compliance and implementation of Nourish digital care management system used in adult social care. Previous improvement requirements relating to personal plan reviews, health and safety records, supervision, appraisal and training were recorded as achieved. However, further assurance is required regarding the frequency of Regulation 73 visits, completion of quality questionnaires and whether audit actions consistently reduce recurring risks. The quality assurance policy did not clearly specify audit frequencies or tools for care plans, risk assessments and behavioural support plans.

The provider evidenced investment in electronic care, HR, training and incident-monitoring systems, alongside a two-year premises improvement plan. However, substantial agency expenditure, the maintenance backlog and outstanding fire-safety works require continued provider oversight to ensure resources support sustainable improvements.

Care Staffing numbers met assessed needs during the inspection, recruitment files were satisfactory and mandatory training compliance was high. Nevertheless, significant and increasing reliance on agency workers across day and night shifts presents a concern regarding continuity of care. The provider has explained the pressures and introduced recruitment measures, while recurrent agency personnel provide some consistency. This should remain an area for improvement and monitoring rather than non-compliance at this stage.

The service is well led and supports people well, with evidence of responsive leadership and improvement. Further strengthening is required in audit assurance, RI visit evidence, workforce stability and oversight of recurring risks.

Areas identified for improvement

Where we identify **Areas for Improvement** but we have not found outcomes for people to be at immediate or significant risk, we discuss these with the provider. We expect the provider to take action and we will follow this up at the next inspection.

Where we find outcomes for people **require significant improvement** and/or there is risk to people's well-being we identify areas for **Priority Action**. In these circumstances we issue a Priority Action Notice(s) to the Provider, and they must take immediate steps to make improvements. We will inspect again within six months to check improvements have been made and outcomes for people have improved.

CIW has no areas for improvement identified following this inspection.

CIW has not issued any Priority action notices following this inspection.

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